

ne Em. Emphasis
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[NAME]
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Name: [NAME] SALAMA ABDE!

'L HADY MOHAMED.
Date: [DATE] [DATE]

IRLAND MRA OF THE [HOSPITAL]

TECHNIQUE:

[NAME] stu.
[HOSPITAL] av,

coronal [NAME],

Axial 11, T2and FAIR Wis,
TorMea,

MRI FINDINGS:

Right frontal periventricular area of diffusion rest
diffusion Wis and low ADC value: It also di
© Perivent

striction is seen, displaying high signal in
isplays high T2 and [HOSPITAL] signal intensities

ular thin sheets of high T2 and [HOSPITAL] signal are seen,

* Few bilateral frontal

and parietal small periventricular and subcortical foci of high T2 and
[HOSPITAL] signal are seen neither surrounded by edema nor exerting mass effect.

Prominent cerebral ventricles and mildly dilated cortical sulci and extra axial [HOSPITAL] spaces.

* Normal size and parenchymal signal intensity pattern of the cerebellum and brain stem,

* No shift of the midline structures.

* No intra or extra axial \$.0.L5

© <NRP>-axial collections.

Intact cranio-cervical junction.

[HOSPITAL] findings

* Mild diffuse atherosclerotic changes show

ving mild wall irregularities and intimal thickening

* Otherwise, patent normal caliber internal carotids and cerebral arteries. No evidence of malformation.

both anterior and middle stenotic segments, vessel occlusion or vascular

'+ Hypoplastic right vertebral artery.

Patent average caliber of the basilar and both posterior cerebral arteries with no evidence of vessel occlusion or stenotic segments:

OPINION:

© Right cerebral acute non hemorrhagic infarct.

* Small artery disease with bilateral cerebral ischemic foci.

n involutional changes.

Much Obligated

Dr. [NAME], [LOCATION]